جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : امنيه سيد رضوان محمد
المحافظة : القاهره **الحي / المدينة :** السيده زينب **العنوان :** 11 ش بدوى
رقم البطاقة : 28802060100301 **رقم التليفون :** 01125979194 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 06-02-1988 **المهنة :** بدون عمل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: امنيه سيد رضوان محمد
National ID: 28802060100301

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

جوستافروسي انترناشونال

إسم المريض ثلاثي: .....
رقم الملف: .....

تقريـر طبـي

السيد/.....

تحية طيبة وبعد،،

إيماء للطلب المقدم من ..... بشان تقرير طبي للمريض:.....

بقسم.....رقم طبي .....

نتشرف بأن نوضح فيما يلي تقريرا عن حالته :

تاريخ وساعة الدخول..... .....تاريخ وساعة الخروج.....

وبعد الكشف والفحوصات والمتابعة وجد انه:

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تحريرا في

الوقت: ..... **التاريخ :** / /

الطبيب المعالج

.....

يعتمد ،،


جمهورية مصر العربية
بطاقة تحقيق الشخصية
امنيه
سيد رضوان محمد
٢٠ش مهدلى-ارض يعقوب
السيدة زينب - القاهره
٢٨٨٠٢٠٦٠١٠٠٣٠١
K03910467


٢٨٨٠٢٠٦٠١٠٠٣٠١
٢٠٢٤/٠٥
يدون عمل
أنثى
مسلمة
متزوجة
محمد سيد محمد حسن
البطاقة سارية حتى ٢٠٣١/٠٥/٢٩
١

Name: امنيه سيد رضوان محمد **MR. :** 5771	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 37-year-old individual with a diagnosis of premenopausal invasive lobular carcinoma (ILC) who underwent upfront left modified radical mastectomy (Lt MRM). The adjuvant plan was outlined as three cycles of chemotherapy (3AC) followed by three cycles of taxane (3 TXT) with placement of a port, then sequential tamoxifen and Zoladex for five years. The first cycle of taxane (C1 TXT) was initiated at a dose of 140 mg with supportive care. The patient has completed one cycle of the TXT 140 mg regimen.

For the second cycle, the patient received C2 TXT 140 mg. Laboratory results were acceptable, and the patient reported diarrhea attributed to vitamin B12 deficiency secondary to diabetes and gaptin use. The patient has completed two cycles of the TXT 140 mg regimen.

The patient, now 38 years old, remains premenopausal and is currently receiving adjuvant radiotherapy. Pathology confirmed ILC with estrogen receptor (ER) 8/8, progesterone receptor (PR) 8/8, HER2 1+, and low Ki-67. Today the patient was started on Zoladex and tamoxifen, prescribed for a three-month supply from the pharmacy. The current plan is to continue Zoladex and tamoxifen as adjuvant endocrine therapy while completing radiotherapy.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : اسماء محمد مصطفي نصر

يعتمد ,

GUSTAVE ROUSSY INTERNATIONAL EGYPT

MDT FORM

Patient Name	امنيه سيد رضوان	Age	37
Sex		Special habits	
Family History	+ve	Comorbidity	HTN, DM
Surgical History		Telephone	01125979194
Menopausal Status	premenopausal	Marital Status	married
ID	28802060100301	Medical ID	2100/2025
Date	18/11/2025	Reason of MDT	New case

Medical Report

Pathology: Bx-> invasive mixed ductal & lobular

TNM

ER 8/8 - PR 8/8 - HER2 +1 - Ki67 3%

S.M: left breast area of altered parenchyma 6 o'clock 39x17x34
Metsworkup: CT CAP >> RT breast soft tissue nodule ??
RT axillary indeterminate LN
RT lung lobe nodular shadowing >> free
Bulky uterus >> free
Upper pole spleen >> calcified lesion
Sacroilitis for bone scan
dorsal scoliosis

Recommendations: RT axillary node biopsy / CESM to assess bilaterality

Doctor orders: Post Lt MRM (Free margins - Free nipple areola - LN 1/7 - T3N1

MDT DECISION	3 AC + 3 DOCETAXEL PORT ZOLADEX/ TAMOXIFEN X 5YEARS

Name	د/ خالد عبدالعزيز	د/ هاشم عبدالرحمن	د/ ابو بكر حجازي
Signature		Dr Hash 18/11/2025	

USTAVE OUSSY INTERNATIONAL

Patient Name	: الأستاذه / امنيه سيد رضوان محمد	Request No.	: 13008418
Patient No.	: 28802060100301	Request Date	: 28/07/2025
Path No.	: SR-1032/25	Verification Date	: 31/07/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data
Left breast mass.

Nature of specimen
Block &slide revision

Gross examination
One block labelled by (25021109) &one slide were received.
IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
* Histological type: Invasive mixed lobular & duct carcinoma.
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade 2
* Lobular intraepithelial neoplasia: Positive
* Angioinvasion: Couldn't be detected in the received biopsy
*Perineural invasion: No
*Microcalcification : Absent
• IPT (Positive control showed appropriate reactivity) :
* ER: Strong nuclear reaction in 95 %tumor cells (Allred score 8/8)
* PR: Strong nuclear reaction in 80 % tumor cells (Allred score 8/8)
* HER2: Faint incomplete membrane reaction in <10 % of tumor cells (score 1+)
* Ki 67:3 %

Diagnosis

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

USTAVE OUSSY INATIONAL

Patient Name	: الأستاذه / امنيه سيد رضوان محمد	Request No.	: 13008418
Patient No.	: 28802060100301	Request Date	: 28/07/2025
Path No.	: SR-1032/25	Verification Date	: 31/07/2025
Sex	: Female	Financial Category	: جوستاف روسي - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Left breast mass, Block &slide revision:
Mixed Invasive lobular and duct carcinoma, Grade 2, with positive intraepithelial lobular neoplasia .

IPT:
* ER: Positive (score 8/8).
* PR: Positive (score 8/8).
* HER2: (score 1+).
* Ki67: 5 %
* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr.Walaa Ibrahim Ghoneim
Dr.Heba El-Shafeey

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient ID: 88399	Patient Name English: Omnia Said Radwan Mohamed
Age: ٣٧ سنة, ٧ شهر	Report Date :2025/09/11 11:34:24 AM

Name: Omnia Sayed Radwan

Date: 11/09/2025

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: Known case of left breast pathological proven invasive mixed lobular and duct carcinoma (+ve ER, +ve PR and -ve HER2 and Ki67= 3%)

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild nodular background enhancement

ACR: D (Extremely dense)

LEFT BREAST:

- **Low energy (Non contrast) images**: Left breast shows posterior 1/3 UOQ ill-defined spiculated mass lesion measures about 16 x 16 mm and 11 cm from the nipple associated with architectural distortion.
- **Recombined (Post contrast) images**: Multiple scattered pathological heterogenous mass and non-mass enhancement involving the whole breast and reaching the nipple with most anterior satellite about 12 mm from the nipple.
  - Upper outer (index lesion) (most lateral) (located at posterior 1/3), measures about 11 x 10 mm and about 11 cm from the nipple.
  - Lower inner (most medial) (located at posterior 1/3), measures about 11 x 10 mm and about 7 cm from the nipple.
  - Upper central (most upper) (located at posterior 1/3), measures about 10 x 10 mm and about 12 cm from the nipple.
- Normal skin thickness.
- **Complementary US**:
  - Most lateral one (index lesion) located at 2:00 clock and measures about 16.7 x 14.5 mm and about 8.5 mm from the skin.
  - Most medial one located at 8:00-9:00 clock and measures about 16.6 x 5.4 mm and about 4 mm from the skin.
  - Most upper one located at 11:00 clock and measures about 10.9 x 7.3 mm and about 15 mm from the skin.
  - Most lower one located at 6:00 clock and measures about 12.1 x 10.1 mm and about 12 mm from the skin.
  - Most anterior one measures about 7.2 x 5.7 mm and about 12.7 mm from the nipple.

- Few enlarged pathological axillary lymph at level I with thickened cortex, largest one measures about 15.7 x 6.4 and focal cortical thickening measures about 4.4 mm.

RIGHT BREAST:

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images** show no pathological uptake.
- Normal skin / nipple / areolar complex.
- **Complementary ultrasound:**
  - Right nonspecific axillary lymph nodes.

IMPRESSION:

- **LEFT BREAST:** (BIRADS = 6) Left multicentric malignant process reaching the nipple with pathological axillary lymph nodes as described
- **RIGHT BREAST:** (BIRADS 2= benign)

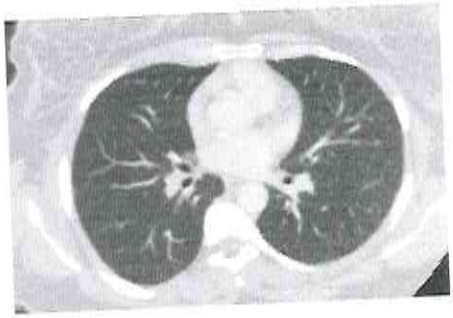
Best regards,

Dr. Hedaya Nour, MD

جوستاف روسي انترناشونال مصر

Patient Name: Omneya Sayed Radwan Mohamed	Date : 26/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 37 Year(s)
Medical ID: 2100/2025 1720 1000	Pat. ID: 110682

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

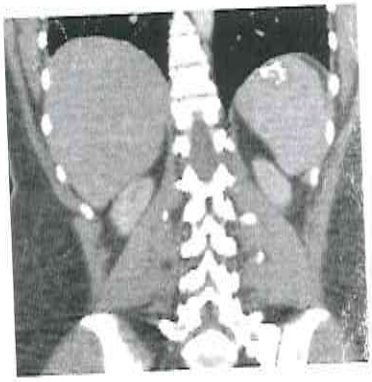


TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. I.V contrast administration.

FINDINGS:

CECT CHEST:

- Right breast shows retroareolar soft tissue nodule measures about 16 x 19 mm.
- Bilateral axillary lymph nodes, more prominent on left side, largest one is rounded in shape measures about 13 x 13 mm.
- Right lower lung lobe subpleural nodular shadowing is seen measures about 3mm.
- No cavitary lesions or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.
- Right central venous line.



CECT ABDOMEN AND PELVIS:

- Upper pole of spleen calcified lesion is seen measures about 16 x 18 mm.
- The liver is average in size showing homogenous CT density with no enhancing focal lesions or biliary radicles dilatation.
- The gall bladder shows average wall thickness. Its cavity shows no dense stones.
- Normal CT appearance of the pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic pathologically enlarged lymph nodes.
- No free collection.

- Bulky uterus.

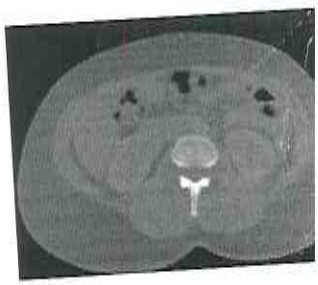
THE SCANNED SKELETON:

- Tiny sclerotic focus is seen at LV2 verteb ral body.
- No vertebral structural collapse or extra-osseous soft tissue component.
- Bilateral sacroiliitis.
- Dorsal scoliosis with its angle to right side.

- **Opinion:**

- ***Right breast shows retroareolar soft tissue nodule***
- ***Bilateral axillary lymphadenopathy, more suspicious at left side.***
- ***Right lower lung lobe subpleural nodular shadowing.***
- ***Tiny LV2 vertebral body sclerotic focus.***
- ***Upper pole of spleen calcified lesion.***
- ***Bulky uterus.***
- ***Bilateral sacroiliitis.***
- ***Dorsal scoliosis***

Dr.Heba El Balshy.MD
Lecturer of Diagnostic and Interventional Radiology,
NLI-Menoufia Univeristy



Name : Omnia Said Radwan Mohammed
Branch : Dokki

Date : 06-09-2025 10:28
Medical No. : 195284

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History:

- 37-year-old lady, with history of left breast cancer, referred for metastatic workup.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions that may account for metastatic bone disease could be seen allover the surveyed skeleton.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTER